Vaccines for Children (VFC) Program Evaluation:

Assessing Effectiveness and Addressing Health Disparities in the North Georgia Health District 1-2

Course: HPRB 7470E-Summer 2025

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- Roster VFC.docx

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Program Overview – What is VFC?

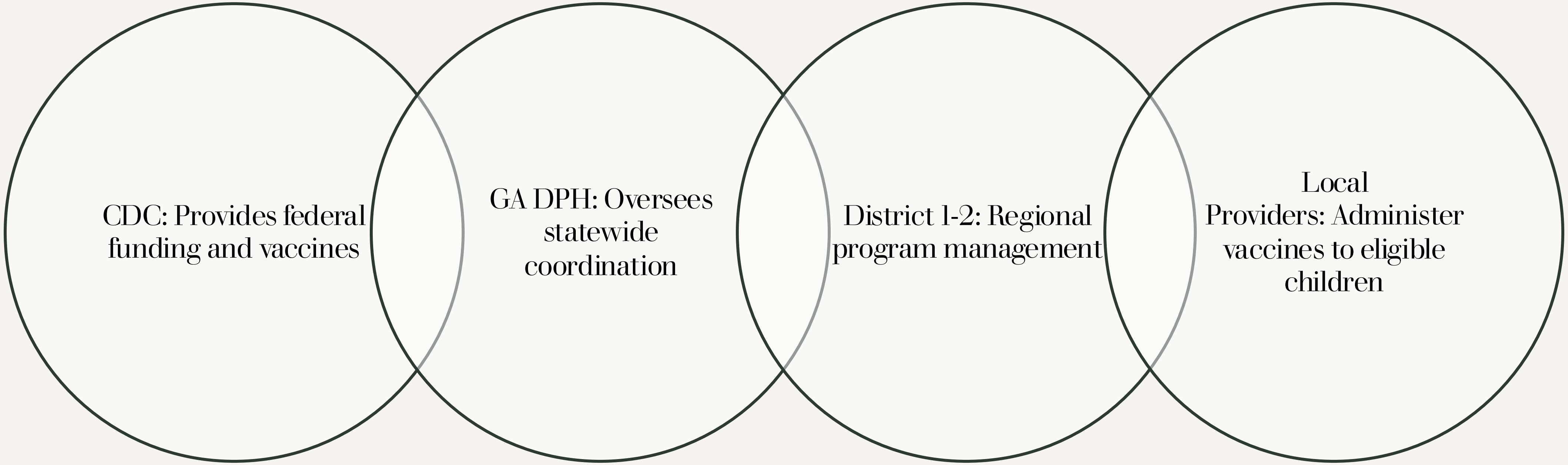
- **Program Name:** Vaccines for Children (VFC) Program
- **Agency:** North Georgia Health District 1-2

- **Program Rationale:** VFC was created in response to the early 1990s measles outbreak, which highlighted that children were not being vaccinated due to high costs. It addresses health disparities and barriers faced by low-income families by providing access to 19 vaccines. The program aims to ensure high vaccination rates among eligible children and prevent missed doses due to cost or delayed referrals.

- **Establishment:** Created on August 10, 1993, through the Omnibus Reconciliation Act, and became fully operational by October of that year.

- **Funding:** Federally funded. The Georgia Department of Public Health (DPH) manages funding and sets policy for Georgia's Health Districts in compliance with HHS and CDC rules.

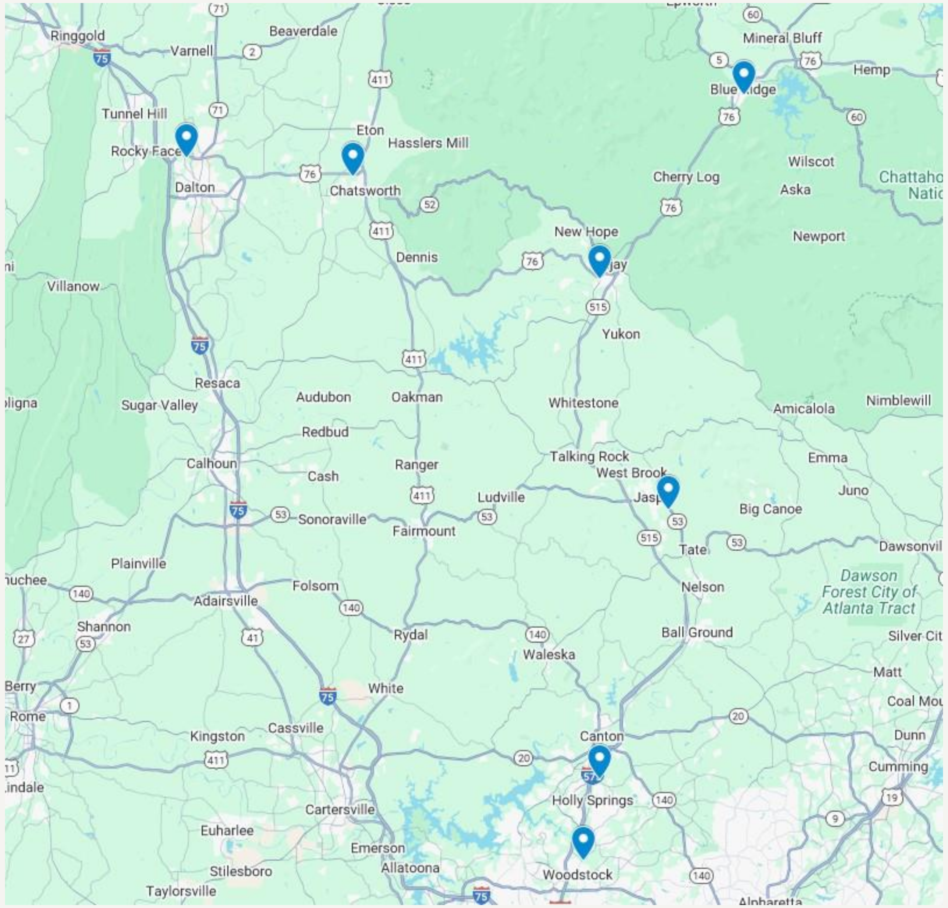
Program Operations



Program Locations

Locations in North Georgia Health District 1-2: The program operates in seven strategically placed locations, across the 6 counties of the health district, to enhance equitable access:

- Cherokee County Health Department
  - South: Woodstock
  - North: Canton
- Fannin County Health Department: Blue Ridge
- Gilmer County Health Department: Elijay
- Murray County Health Department: Chatsworth
- Pickens County Health Department: Jasper
- Whitfield County Health Department: Dalton



Program Environment & Challenges

Current Features: VFC is federally funded, staff receive two training courses, and 7 locations are strategically placed for equitable access across the 6 counties served by District 1-2.

Health Dimensions: The program aims to increase herd immunity and reduce vaccine-preventable diseases in the region.

Economic Dimensions: While specific economic data for the North Georgia Health District 1-2 counties were not provided, the VFC program specifically targets children from low socioeconomic backgrounds.

Social Dimensions: Challenges exist due to transportation disparities.

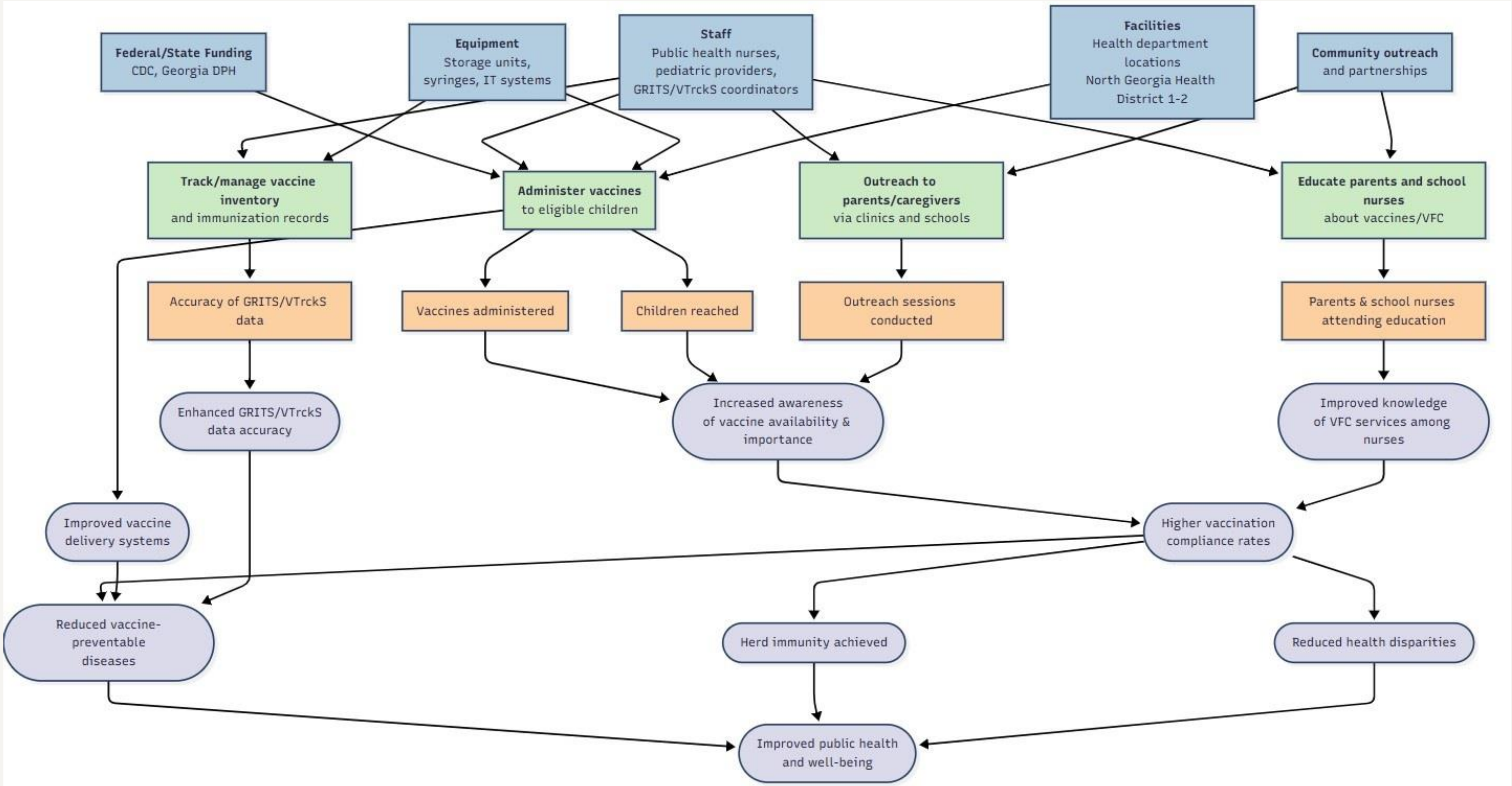
Challenges Identified

Transportation barriers: Living far in rural, or lack of means of transportation.

Difficulty in outreaching families of eligible children, particularly undocumented children.

Information dissemination: Lack of understanding that no child will be turned away for inability to pay for vaccines.

Vaccine Hesitancy by families to have direct contact with the government and Stigma of vaccines


Federal/State Funding
CDC, Georgia DPH
Equipment
Storage units, syringes, IT systems
Staff
Public health nurses, pediatric providers, GRITS/VTrckS coordinators
Facilities
Health department locations
North Georgia Health District 1-2
Community outreach and partnerships
Track/manage vaccine inventory and immunization records
Administer vaccines to eligible children
Outreach to parents/caregivers via clinics and schools
Educate parents and school nurses about vaccines/VFC
Accuracy of GRITS/VTrckS data
Vaccines administered
Children reached
Outreach sessions conducted
Parents & school nurses attending education
Enhanced GRITS/VTrckS data accuracy
Increased awareness of vaccine availability & importance
Improved knowledge of VFC services among nurses
Improved vaccine delivery systems
Higher vaccination compliance rates
Reduced vaccine-preventable diseases
Herd immunity achieved
Reduced health disparities
Improved public health and well-being

Key Interest Holders & Their Role in Evaluation

North Georgia Health District 1-2:	Oversees program implementation and needs to understand effectiveness and reach. This is a key decision-maker and coordinator of the VFC program locally.
Pediatric Clinics/Providers:	Directly engage with the target population and administer VFC vaccines. They are interested in improving vaccine delivery systems and provide valuable insight into front-line implementation issues.
School Nurses/School Districts:	Promote vaccinations among eligible children and desire higher vaccination compliance.
GRITS/VTrckS System Coordinators:	Manage immunization and inventory tracking. They ensure data accuracy and improve reporting systems, providing essential infrastructure for tracking and evaluating data.
Parents/Caregivers of Eligible Kids:	Engage with and use VFC services, concerned with access and trust in vaccine programs. Community-based organizations can facilitate discussions with client families to verify themes around access and outreach barriers.
Public Health Officials (GDPH, CDC):	Engage in dialogues to interpret findings in the context of regional trends and policy priorities, ensuring alignment with broader public health goals.
Grant Interest Holders & Program Administrators:	Review tailored data summary reports to understand how evaluation supports program improvement and future grant justifications.

Evaluation Purpose

Assess the effectiveness of the VFC program in increasing vaccine rates and reducing disparities among low-income children in North GA District 1-2.

EVALUATION QUESTIONS & MEASURES TABLE

Question Type	Evaluation Question	Indicators	Data Sources	Measures
Process	How effectively are stakeholders engaged in VFC planning?	Number stakeholder meetings, Percentage of key partners involved	Meeting records, sign-in sheets	Frequency of meetings, attendance percentage
Process	How many eligible children received vaccines?	Numberof vaccines administered, % timely GRITS entries	GRITS/VTrckS records	Percentage of eligible children vaccinated
Outcome	Has VFC reduced SES disparities in vaccination coverage?	Vaccination rates by SES and race/ethnicity	GRITS/VTrckS, Census data	Percentage difference in vaccination coverage between groups

Evaluation Design: Quasi-experiment

Rationale:

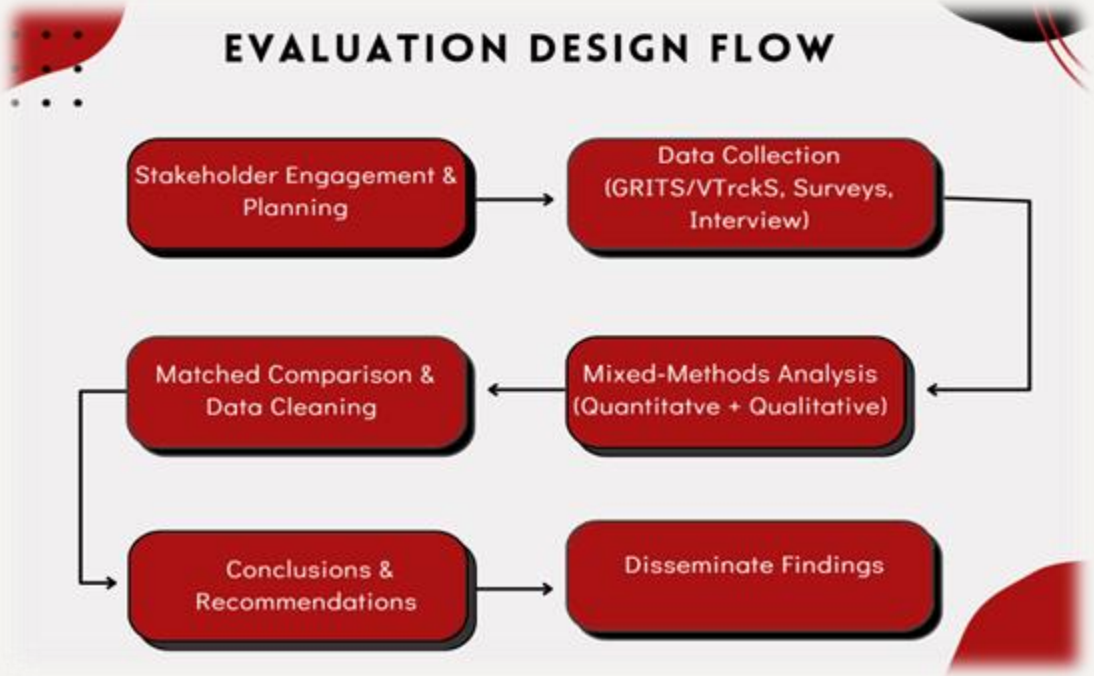
- Ethical and feasible for real-world programs like VFC where randomization is not possible.
- Allows evaluation using existing GRITS/VTrckS and survey data.
- Provides strong evidence by comparing matched groups without withholding vaccines.

Strengths:

- Real-world generalizability.
- Ethical approach – no denial of services.
- Stronger evidence compared to descriptive studies.
- Applicable to public health practice using existing data.

Limitations:

- No random assignment → possible confounding.
- Lower internal validity than RCTs.
- Requires advanced statistical methods to control for biases.



Evaluation Design & Data Collection

Key Indicators & Measures:

- **Vaccine Access Equity:** SES-based vaccine distribution measured by the percentage of children vaccinated in low-income zip codes.
- **Stakeholder Involvement:** Frequency of communication with the CDC/ GA DPH measured by the number of meetings or formal communications per quarter.
- **Data System Performance:** Data completeness and accuracy measured by the percentage of VFC participants with complete immunization records in GRITS/VTrckS.
- **Data Sources:**
  - Key informant interview(Ashley Deverell, RN, BSN, Infectious Disease Dir., North GA Health District 1-2).
  - GRITS/VTrckS data (vaccine tracking, SES, demographics)
  - CDC/GA DPH Budget & financial reports
  - Program operational records and annual reports.
  - Surveys (suggested for future analysis) with parents/caregivers.
- **Analysis Methods:**
  - **Qualitative:** Thematic analysis for interview data (e.g., vaccine hesitancy, transportation barriers).
  - **Quantitative:** Descriptive statistics for GRITS/VTrckS data (total number/percentage of children vaccinated). Inferential statistics for GRITS/VTrckS data (disparities across SES and demographic groups). Geographic Information System (GIS)Mapping of or GRITS/VTrckS data (visualizing vaccine coverage by zip code and SES).

Key Findings & Interpretations

- Finding 1: Outreach Challenges Persist
- Finding 2: Positive Impact on Immunization Coverage
- Finding 3: Persistent SES Disparities
- Finding 4: Uneven Stakeholder Engagement

Finding 1: Outreach Challenges Persist

Interpretation

- Key informant interview identified outreach-related challenges such as transportation, vaccine hesitancy, and lack of program/vaccine knowledge, hindering equitable vaccine access.

Connection to Goals

- Suggests that while outreach is ongoing, significant barriers remain for equitable vaccine access, especially in marginalized populations, highlighting the need to improve outreach strategies to meet the goal of reducing health disparities.

Finding 2: Positive Impact on Immunization Coverage

Interpretation

- GRITS/VTrckS data show a rise in immunization coverage across all six counties in the North Georgia Health District 1-2, indicating VFC's positive impact and timely vaccine delivery. If this trend continues, herd immunity is more likely to be achieved.

Connection to Goals

- Demonstrates progress toward the program's outcome of high immunization coverage within the district.

Finding 3: Persistent SES Disparities

Interpretation

- Key informant interview identified outreach-related challenges such as transportation, vaccine hesitancy, and lack of program/vaccine knowledge, hindering equitable vaccine access.

Connection to Goals

- Suggests that while outreach is ongoing, significant barriers remain for equitable vaccine access, especially in marginalized populations, highlighting the need to improve outreach strategies to meet the goal of reducing health disparities.

Finding 4: Uneven Stakeholder Engagement

Interpretation

- Thematic analysis shows strong collaboration with state-level public health partners but limited direct engagement with grassroots community members. This may weaken trust and reduce the program's ability to identify culturally relevant strategies for underserved populations.

Connection to Goals

- Connection to Goals: VFC partially meets its goal of maintaining relations with interest holders, but a lack of local member representation could contribute to the persistent health disparity gap, indicating a need for stronger local engagement to meet stakeholder involvement and equity goals.

Recommendations for Program Improvement

Recommendation 1: Enhance Grassroots Community Engagement

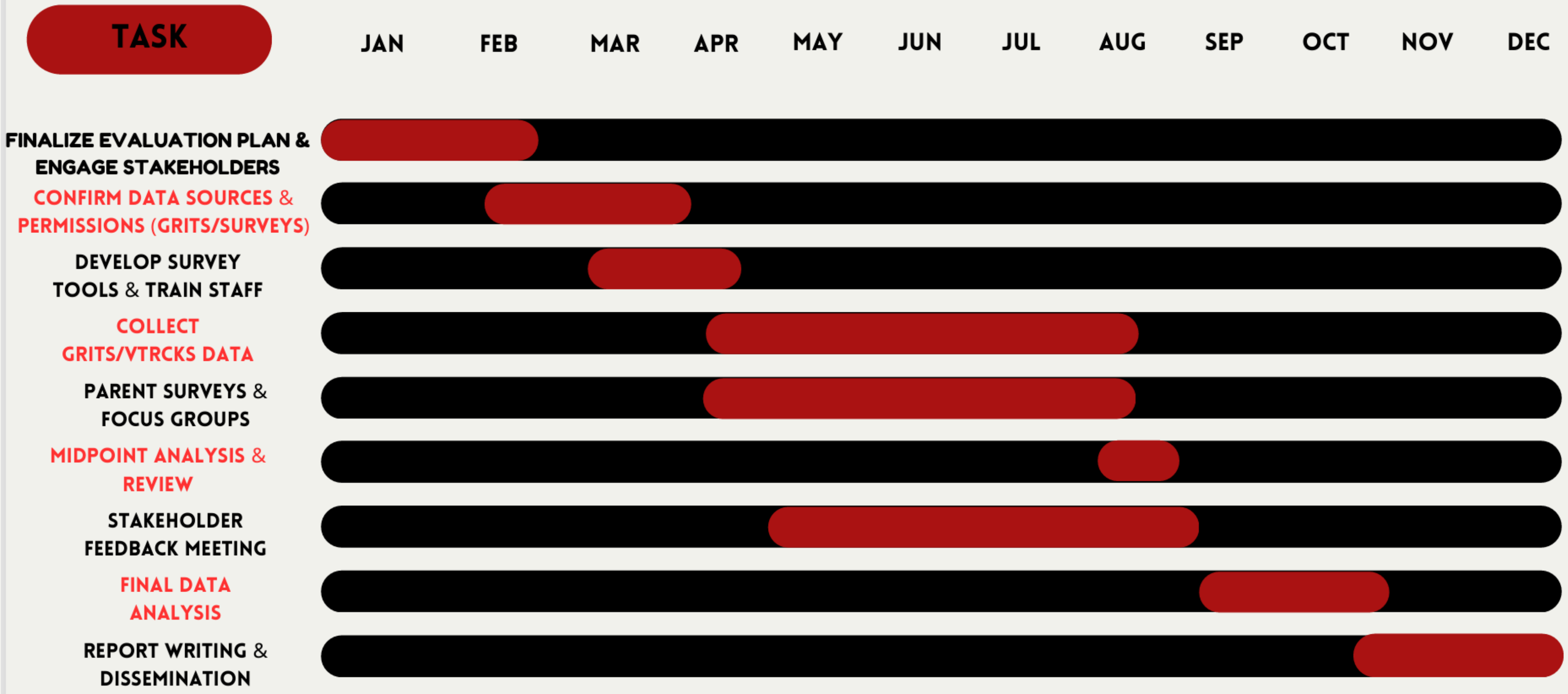
- **Conclusion Addressed:** Limited direct engagement with grassroots community members.
- **Recommendation:** Develop and implement a structured plan for consistent, culturally sensitive engagement with local community leaders and organizations in low-SES areas. This could involve establishing community advisory boards, holding regular town halls, or partnering with trusted community-based organizations for outreach.
- **Expected Benefit:** This will build trust, identify culturally relevant strategies, address vaccine hesitancy and transportation barriers more effectively, and provide vital insights to close health disparity gaps, ultimately improving equitable vaccine access.

Recommendation 2: Strengthen Targeted Outreach and Education

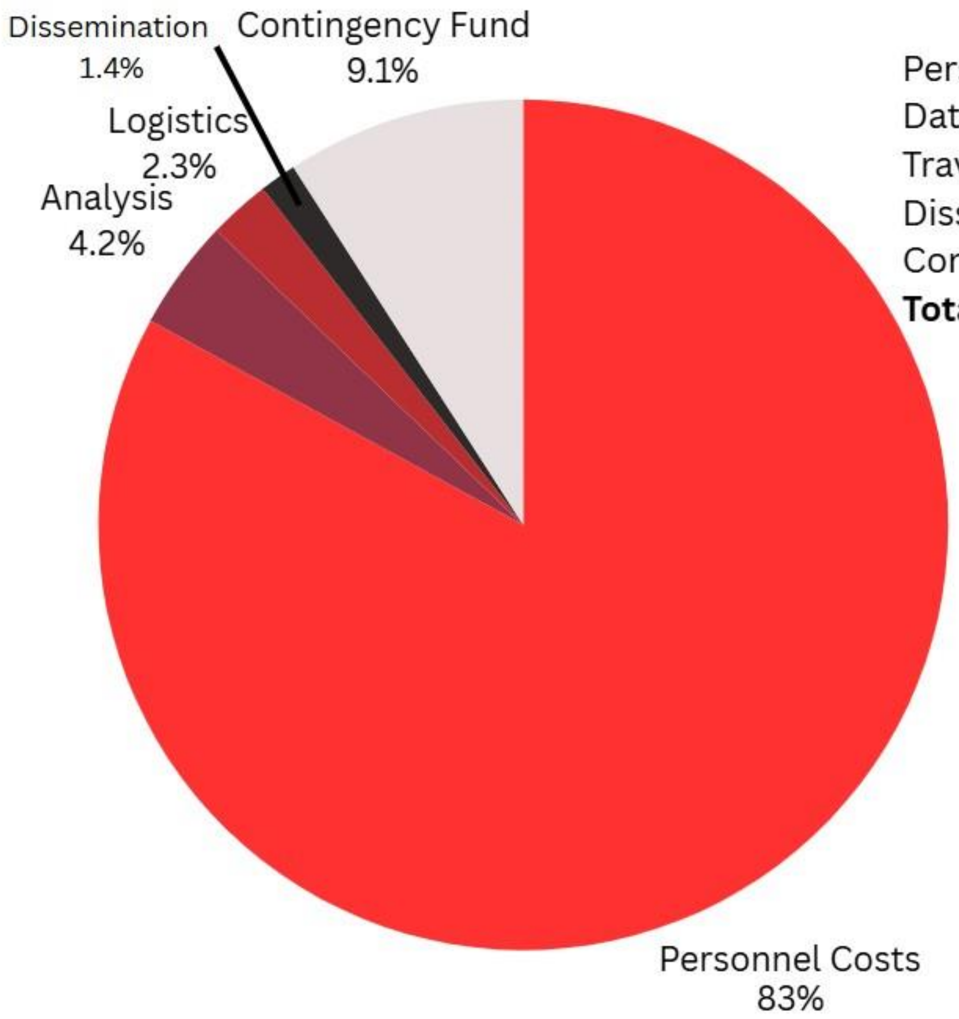
- **Conclusion Addressed:** Persistent lower vaccination rates in lower-SES households and challenges with awareness and understanding of the program.
- **Recommendation:** Utilize GIS mapping from GRITS/VTrckS data to identify specific geographic areas with low vaccination rates and high SES disparities. Implement targeted outreach campaigns in these areas, including mobile clinics, educational materials in multiple languages, and direct communication emphasizing that no child will be turned away due to inability to pay.
- **Expected Benefit:** This will strategically focus resources, increase program awareness and accessibility for underserved populations, and directly contribute to closing the immunization gap and achieving higher herd immunity levels.

VFC PROGRAM EVALUATION

GANTT CHART



VFC Program Evaluation Budget - Health District 1-2



Personnel Costs :	$20750.00
Data Collection/Analysis:	$1050.00
Travel & Logistics:	$585.00
Dissemination:	$350.00
Contingency Fund:	$2273.50
Total Budget:	**$25008.50**

Data Analysis and Dissemination

Quantitative Analysis

- Analyze GRITS/VTrckS data for vaccine timeliness and SES-based trends.

Qualitative Analysis:

- Code interviews and focus groups to identify barriers, trust issues, and community needs.

Dissemination plan :

- Target audiences: GA DPH, CDC, Program staff, community clinics, parents, and policy makers.
- Use of results: improve outreach strategies, support mobile clinic grant reapplication, inform policy.

Conclusion

Enhance Targeted Outreach: Develop and implement strategies to identify and engage eligible children, particularly undocumented children, addressing hesitancy towards government contact.

Strengthen Communication on Cost-Free Vaccines: Clearly and consistently communicate that no child will be turned away due to inability to pay for vaccines.

Address Transportation Barriers: Explore and implement solutions for transportation disparities that hinder access to vaccination sites.

Improve Stakeholder Collaboration: Foster more consistent and effective engagement with all key interest holders, including public health officials, pediatric providers, and community organizations.

Leverage Data for Equity: Utilize GRITS/VTrckS data and GIS mapping for ongoing monitoring of vaccination coverage by SES and geographic area to identify and address persistent disparities.

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AI Use Disclosure

ChatGPT (OpenAI) was used to assist in generating sample charts and diagrams from project data, providing example visuals to work from. Mermaid was used to draft flow diagrams, and Google Maps was referenced for location details of North Georgia District 1 and 2. Canva was then used to create, edit, and finalize all graphics and visual elements. All AI-assisted or tool-generated content was reviewed and finalized by the project team to ensure accuracy and alignment with project requirements.

Appendix

- Key Informant Interview Summary (summarized in a slide; PDF or docx file)
- Group 3 - Key Informant Interview notes and Summary.docx
- Logic Model (also shown in a single slide; PDF or docx file)
- Logic Model .docx
- Detailed Budget Table (summarized briefly in presentation; Excel of PDF file)
- Refined Evaluation Budget for VFC Program (North Georgia District 1-2) .docx
- Timeline (summarized briefly in presentation as Gantt-style chart; Excel of PDF file)
- VFC Program Evaluation .pdf
- Timeline.docx
- Presentation Slides (same as what was used to create the MP4 presentation; pptx or PDF file)
- Vaccines for Children (VFC).pptx
- Reference list only if not included in the presentation slides (docx or PDF file)
- Roster VFC.docx
- Speakers who represented the slides